

LIPOMA

A 52 year old male Mr.Ramasamy, a manual labourer, from Tondiarpet , came with **chief complaints** of

Swelling in the left side of nape of the neck for the past 4 years.

History of presenting illness:

Patient was apparently normal 4 years back , then he noticed a swelling in the left side of nape of the neck

Which was insidious in onset ,

Started as a small swelling of almost a size of almond

Gradually increased in size to attain the present size.

Not associated with pain.

No H/o Fever

No H/o Trauma

No H/o any other swelling

anywhere in the body.

No H/o loss of weight & appetite.

No H/o restriction of movements.

No H/o secondary changes over the swelling.

Past history / Personal history / Treatment history

General examination

Vital Signs

Local examination

Inspection – Solitary swelling in the left side of nape of the neck 8*5 cm ovoid in shape

Extent :

Upper limit: hair line

Lower limit: 2cm above the C7 spine.

Medial limit: just crossing midline

Lateral limit: 2cm from mastoid.

Margins well defined

Surface smooth

Skin over the swelling is normal

No punctum, No visible pulsation

No scar. No sinus, no dilated veins

No secondary changes over the swelling

Surrounding area is normal

Palpation:

Not warm, Not tender

Inspectory findings like site size shape extent are confirmed

Margins are well defined

Surface lobulated, Soft in consistency

Skin over swelling is pinchable

Plane of the swelling is subcutaneous

Freely mobile in all direction

Slip sign negative

Fluctuation negative

Not compressible or reducible

Not pulsatile or expansile

Regional lymph nodes are not palpable

Other system examination:

Provisional diagnosis :

A case of soft tissue tumour most probably LIPOMA

Investigations

General.

Specific: FNAC

Trucut Biopsy

Treatment :

Excision of the lipoma

HYDROCELE

Mr.Elumalai,54 years male,Teynampet who is a Cab driver came with **chief complaints** of

Swelling in the left side of the scrotum for the past 5 years.

History of Presenting Illness

Patient was apparently normal before 5 years

He developed a swelling in the scrotal region

Insidious in onset

Slowly progressive to attain the present size

No aggravating and relieving factors

Does not reduce on lying down

No H/o pain

No H/o trauma

No H/o fever with chills and rigor

No H/o heavy weight lifting

No H/o loss of weight

No H/o loss of appetite

No H/o burning micturition

No H/o any other swelling

Past History & Personal history

Family history

Treatment history

General Examination

Vital Signs

Local examination - Inspection

Patient examined in standing position

Left sided scrotal swelling

12*10 cm , Oval in shape

Extends from pubic symphysis to bottom of the scrotum

Skin is stretched and shiny

Rugosity is lost

No dilated veins seen over the scrotum

No ulcer or discharge from the swelling

Cough impulse – negative

Penis in midline

Pubic Hair not seen

Palpation

Patient in standing position

Not warmth ,Not tender

Left sided swelling seen over the scrotum

15*13 cm ,Oval in shape

Extends from root of the scrotum to base of the scrotum

Not reducible

Able to get above the swelling

Fluctuation positive

Trans-illumination positive

Cystic in consistency

Testis not palpable –LEFT SIDE

Right side testis normal

OTHER SYSTEM EXAMINATION

Diagnosis

LEFT SIDED PRIMARY VAGINAL HYDROCELE

Specific Investigation - Ultrasound scrotum

Routine Investigation

Treatment

Jaboulay's method – eversion of sac

Lord's plication

SEBACEOUS CYST

Mr.Lakshmanan 34/M from Royapuram who is salesman by occupation came with **Chief complaints** of

Swelling in the right side of the forehead for past 3 years

HISTORY OF PRESENTING ILLNESS

The patient was apparently normal before 3 years. Then he noticed a pea sized swelling in the right side of forehead which has progressed to attain the presentsize

Insidious onset ,

Gradually progressive ,

Not associated with pain or any discharge

No h/o fever

No h/o any other swelling in the body

Past history

Personal history

Family history

General examination

Vital signs

Local examination

Inspection :

A single swelling on the right side of the forehead

approximately 2×2 cm

spherical in shape

surface appears to be smooth

margins – well defined

Extent -: upper limit - 1 cm below hairline

lower limit - 3 cm above right eyebrow

medially - 4 cm from midline

laterally - 7 cm from the ear

Skin over the swelling : punctum seen

No hair follicles

No scars , no sinus , no dilated veins , no visible pulsation

Palpation

Not warm , not tender

Inspectory findings are confirmed :A single spherical swelling of size 2×2cm with a smooth surface and well defined margins on the right side of forehead .

Consistency – cystic, moulding present

Fluctuation – pagets test positive

Transillumination – negative

Skin -not pinchable

Plane of swelling – arises from the skin

Mobility - freely mobile over the underlying structures

No lymph nodes palpable

Other system examination

Provisional diagnosis:

Sebaceous cyst on the right side of the forehead

Management

Investigations

General

Specific - Excision biopsy

Treatment

Complete Excision of the cyst under local anesthesia

DERMOID CYST

Mrs. Maheshwari 25/F Home maker From Washermanpet

Presenting complaints:

Swelling in outer corner of Right eye for the past 3 years

History of presenting illness:

Apparently normal before 3 years

Swelling in outer corner of Right eye for the past 3 years

Insidious in onset

Initially pea sized ,slowly progressed to attain current size

Not associated with pain

Not associated with any discharge

No h/o fever

No h/o trauma

No h/o any secondary changes over the swelling

No h/o difficulty in opening eyelids

No h/o defective vision , redness , discharge from eyes

No h/o headache, vomiting, seizures

No h/o loss of appetite , loss of weight

No h/o any other swellings in the body

Past history / Personal history / Family history

General examination

Vital signs

Local examination

Inspection:

Solitary hemispherical swelling of size 5 x 5 cm

Above outer canthus of Right eye

Smooth surface,Well defined margin

Skin over swelling is stretched and shiny

Lateral edge of eyebrow lies over the swelling

No punctum over the swelling

No scars, sinuses, dilated veins, ulcerations or visible pulsations

Palpation:

Not warm, Not tender

Hemispherical swelling of size 5 x 5 cm

Surface is smooth, Well defined margin

Skin over the swelling is pinchable, Mobile in both axes

Plane of swelling - subcutaneous

Soft in consistency

Slip sign - negative

Fluctuation test - positive

Transillumination test - negative

Not compressible or reducible

No cough impulse, Not pulsatile

No bony indentation felt

Regional lymph nodes not palpable

Summary

25/F came with a **painless**, slowly progressing swelling over the **outer canthus** of Right eye which on examination was a **subcutaneous, smooth, soft, fluctuant, mobile** swelling with **no punctum** over the surface.

Provisional Diagnosis

A case of dermoid cyst over outer canthus of Right eye

Investigations

Specific

X-Ray skull – AP and Lateral (+/- CT head) & FNAC

General

Treatment

Complete Excision of the cyst

SALIVARY GLAND SWELLING

Mr. Masthan Babu 29/M from chepauk who is a tailor came with

CHIEF COMPLAINTS

Swelling below and behind the left ear- 4 years

History of presenting illness

The patient was apparently normal 4 years back

Then he developed swelling just below and behind the left ear lobule for the past 4 years

Insidious onset, Gradually Progressive

No H/O fever , No H/O trauma

No H/O variation in size of the swelling or pain during meals

No H/O referred pain in the ear

No H/O difficulty in swallowing

No H/O sudden increase in size

No H/O pain over the swelling

No H/O difficulty in opening or closing the mouth or chewing food

No H/O deviation of angle of mouth or difficulty in closing eyelids

No H/O loss of weight, loss of appetite

Past history (recurrence M/c) / Personal history / Family history

General examination

Vital signs

Oral cavity examination

Nicotine stains present

No dental caries ,Tongue moist

Tonsils not enlarged in the left side,Uvula in midline

Local examination

INSPECTION-

A swelling is present in the region of parotid in the left side of

Size 5*3 cm, oval in shape,

Surface appears smooth

Skin over the swelling appears reddish and shiny,

Margins - well defined

No dilated veins over the swelling

Retromandibular groove – obliterated

Ear lobule – lifted

PALPATION

Not warm, not tender, Size 5 * 3 cm

Inspectory findings confirmed, Consistency-firm

Extent-superior- just below ear lobule inferiorly upto left angle of mandible

anteriorly-2 cm from tragus & posteriorly- 1 cm from mastoid process

Not fixed to skin ,masseter or bone

Bimanual palpation- deep lobe is not palpable

Bidigital examination-stenson's duct palpable

FACIAL NERVE - No deviation of angle of mouth

Wrinkles present over forehead

Nasolabial fold – present

Eye closure is normal

Taste sensation – preserved

Examination of mandible- no mandibular thickening

Examination of lymph nodes- Preauricular nodes and neck nodes not palpable

Diagnosis- Pleomorphic adenoma of left parotid gland.

DD - Pleomorphic adenoma / Warthin's tumour Oncocytoma / Mucoepidermoid carcinoma(low grade)

Specific Investigations

FNAC OF LEFT PAROTID GLAND

CT Scan /MRI

Treatment - Superficial parotidectomy

DIABETIC ULCER

Mr.Subramani 58/M from Porur who is a cook came with **Chief Complaints of**

Wound in Left leg for 4 weeks

H/OPI :Patient was apparently normal before 4 weeks

Wound in medial aspect of lower half of left leg for 4 weeks

Started as a blister, burst out to become an ulcer ,insidious in onset, progressive in nature and it has attained present size

H/o yellowish discharge, scanty in amount and foul smelling.

H/o loss of sensation around ulcer

H/o limitation of movements in ankle joint

No h/o pain

No h/o trauma/ fever / burns

No h/o dilated veins

No h/o loss of weight / loss of appetite

Past History

Personal History

General examination

Vitals

Local Examination - Inspection

Site: ulcer in the medial aspect of distal half of left leg, upper limit extending 20 cm from tibial tuberosity and lower limit 4 cm from heel and it is 2 and 7 cm medial to shin of tibia of 20cm×5cm×2 cm

Irregular in shape

Margin: well defined

Edge : sloping

Floor: red granulation tissue

Discharge : scanty serous discharge

Skin hyper pigmented , scaly , edematous, loss of hair , brittle nails.

No varicose veins

PALPATION

Skin not warm, not tender

Inspectory findings of site, edge and margin are confirmed

Size 22×5×2 cm with indurated margins

Base: bone

Ulcer is not mobile & Does not bleed on touch

Dorsalis pedis and Posterior tibial artery pulsations are feeble in left leg.

Popliteal pulsations felt in left leg.

Sensation decreased in left foot.

No palpable lymph node

Other system examination

PROVISIONAL DIAGNOSIS - A case of chronic healing diabetic ulcer on the medial aspect of left leg with neuro-vascular deficit.

DD - Venous ulcer / Arterial ulcer

SPECIFIC:

Discharge C/S ,Edge biopsy

X-ray left leg and foot

BASIC:

Complete hemogram

Blood sugar, HbA1C,
lipid profile

Urine examination
for sugar and ketone

Arterial doppler

Renal function test

Fundus examination

Electrocardiogram

Chest X-ray

HbsAg , HIV-ELISA

TREATMENT

Lifestyle modification - diet plan / exercise / weight reduction

Control of blood sugar- insulin, hypoglycemics

Wound debridement, cleaning and dressing of wound

Appropriate Antibiotics Split skin graft

ADVICIE:

Foot care, MCR footwear, Screening for retinopathy, nephropathy& neuropathy

SOFT TISSUE TUMOUR

Mr. Kannadasan 57 yrs/M from Kallakurichi a Farmer with **CHIEF COMPLAINTS**

Swelling in the Back for past 3 years

History of Presenting illness

Patient was apparently normal before 3 years then noticed a swelling in right side back which was in a pea nut size when he first noticed then it increased in size and it was operated after 5 months in a private hospital and postoperatively he denied radiotherapy. Biopsy details are not known.

H/o swelling in the back for past 8 months

which he noticed while bathing ,insidious in onset

which was initially small in size & it rapidly grown to attain present size

associated with pain for 2 months - Continuous,Dull aching type

No aggravating and relieving factors,Not radiating

No H/o Secondary changes like softening, ulceration, fungation

No H/o discharge from the swelling

No H/o restriction of movements

No H/o pressure symptoms

No H/o any other swelling anywhere in the body

No H/o fever,No H/o trauma

No H/o loss of weight and appetite

No H/o chest pain , cough, hemoptysis, headache, jaundice, abdominal pain

Past History – As mentioned above

Personal History/Rx history / Family history / General examination & Vital signs

Local Examination - INSPECTION

Single swelling in the right side of the back approx. 35 x 30 cm globular in shape

Extent :upper limit: spine of scapula

lower limit:15 cm from iliac crest

medial limit: just crossing 1cm frommidline

lateral limit:6 cm from right posterior axillary line

Edges: well defined, Previous surgical scar present.

Surface: appears to be smooth except in areas of scar

Dilated veins present, No sinus & visible pulsations

Skin is stretched and shiny & loss of hair over the swelling

Surrounding area also has scar of size 5 cm lateral to swelling

No other swellings present in the body

Palpation

Warm & tender

Single swelling in the right side of the back Size approx. 32 x 29 x 9 cm

Surface is smooth except in areas of scar, Edges are well defined, Firm to hard in consistency, Not pulsatile

Skin is not pinchable all over swelling

No scar tenderness & discharge

Movement of swelling is restricted in all directions

Plane of swelling: fixed to muscle

Sensation in the surrounding area is normal

Movements of nearby joints is not impaired

Right and left axillary lymph nodes: not palpable

Other system examination

Diagnosis : **RECURRENT SOFT TISSUE SARCOMA** in the right side of back

Clinical Staging rT2bN0M0(Stage 1b)

Specific Investigations - MRI and TRUCUT BIOPSY

For staging workup: CECT chest, USG abdomen, PET scan

Treatment

After investigations, I will restage & subject this to Multidisciplinary team for consensus opinion & present this case

Surgery : As it is very large, WIDE LOCAL EXCISION is difficult. So NEOADJUVANT CHEMOTHERAPY will be given and then will be reassessed for Surgery

ORAL MALIGNANCY

Mr.Thirunavukarasu 43/M from Pudupet a Coolie with Chief Complaints of

Ulcer over left lateral margin of tongue for 6 months.

HISTORY OF PRESENTING ILLNESS:

Patient was apparently normal 6 months back

Developed ulcer over left lateral margin of the tongue

Insidious in onset

Started as a small ulcer opp. to 2nd molar tooth, progressed to attain the present size.

-Associated with pain for past 3 months

- Intermittent in nature

- Pricking type of pain

- Aggravated on taking foods

- No radiation of pain

No h/o difficulty in chewing

No h/o halitosis

No h/o difficulty in swallowing

No h/o discharge

No h/o difficulty in speech

No h/o loss of appetite

No h/o difficulty in opening the mouth

No h/o loss of weight

No h/o of excessive salivation

No h/o trauma

No h/o inability to protrude the tongue

No h/o recent falling of tooth

No h/o deviation of tongue

No h/o hemoptysis , pleuritic chest pain

No h/o alteration of voice

Past history / Personal history (Gutkha/Smoking) / Family history

General examination

Vital Signs

INSPECTION

Lips normal

Nicotene pigmentation on lips

No ulcers / No halitosis / No trismus

Dental formula – 2123 | 2123
 2123 | 2123

Gums normal

Tongue –ulceroproliferative growth **3.5*2cm** involving middle part of left lateral aspect of tongue. Oval In shape with well defined margins and everted edges

Extent - 1 cm from tip of tongue and 1cm from midline

Floor - reddish with necrotic tissue

Mobility of tongue not restricted

Buccal mucosa normal

Floor of the mouth normal

Retromolar trigone not involved

PALPATION:

Tender

Ulcer does not bleed on touch

Inspectory findings confirmed

Base - indurated

Mandible - no thickening / irregularity

LYMPH NODE EXAMINATION: No palpable lymph nodes

OTHER SYSTEM EXAMINATION

Diagnosis - CARCINOMA TONGUE TNM STAGING – T2N0M0 (STAGE II)

INVESTIGATIONS :

Imaging – orthopantogram , CECT neck ,

MRI

Punch biopsy

VDRL

Upper gastrointestinal endoscopy

Treatment

Left Hemiglossectomy

SKIN MALIGNANCY

Mrs. Anthonyammal 68 /F from Pattabiram a manual worker came with C/o

Ulcer in right side of the head for the past 10 yrs

History of presenting illness

Patient was apparently normal 10 yrs back, then she developed a small ulcer over right side of scalp which was initially small which gradually increased and attained the present size

H/o pus discharge from ulcer +

No h/o bleeding.

H/o pain over the ulcer – past 1 month, dull aching, continuous,

No aggravating and relieving factor

not radiating

- H/o itching+
- N/o h/o trauma
- No h/o headache/vomiting/seizures/limb weakness
- No H/o loss of weight or appetite
- No h/o abdominal distension/cough/hemoptysis/bone pain

Past history / Personal history

Family history / Rx History

General examination

Vital signs

Local examination - Inspection

Size 7*7 cm

Shape- Circular,

Extent -3cm away from midline

5 cm above external occipital protuberance

Edge - raised and beaded

Floor of the ulcer is covered with unhealthy granulation tissue & slough

Margin - well defined

Hyperpigmentation + over the surrounding skin

No other swelling / ulcers elsewhere

Palpation

Inspectory findings confirmed

An ulcer of size 7*8 cm over right parietal region of scalp,tender

Does not bleed on touch

firm in consistency,

Raised and beaded edges

well defined margins

Restricted mobility

Base indurated

No surrounding induration

No other ulcers/swelling elsewhere

No palpable lymph nodes

Investigations - Specific

Edge wedge biopsy

CT Brain

Routine investigations

PROVISIONAL DIAGNOSIS

A case of Basal cell carcinoma over the right parietal region of scalp

Staging T3N0M0 (Stage 3)

Differential diagnosis

Squamous cell carcinoma

Keratoacanthoma

Seborrheic keratosis

Treatment

WIDE LOCAL EXCISION(with 1 cm clearance) and flap cover

TESTICULAR TUMOUR

Name: Mr. Ravanaiyaa 35/M from Ponneri, Daily wage by occupation with C/o Swelling in the right side of scrotum for the past 9 months.

History of presenting illness

Patient was apparently normal 3 months back, after which he developed,

A Swelling occurred in the right side of scrotum

Insidious in onset, Progressive in nature

Associated with feeling of heaviness in the scrotum.

Not associated with pain.

No h/o fever, No h/o trauma

No h/o orchidopexy in childhood

H/o breast enlargement for past 3 months, no discharge from the breast

No h/o loss of libido

No h/o any other mass

No h/o vomiting / abdominal pain

No h/o swelling of legs

No h/o breathlessness/hemoptysis

No h/o bony pain

No h/o loss of weight / loss of appetite

Past History

Family History

Personal History

General Examination & Vitals

Local Examination of Scrotum - Inspection

A swelling of size 16x10cm is seen in the right side of the scrotum

Elliptical in shape

Extent is from root to the bottom of scrotum.

Skin over the swelling is normal, No scar, sinus or dilated veins

No visible cough impulse.

Scrotal skin rugosity reduced

Pubic hair shaved, Penis in midline

Palpation

Not warm, not tender.

Inspectory findings are confirmed.

Hard in consistency.

Fluctuation – Negative

Transillumination – Negative

Able to get above the swelling.

Comparatively right testis feels heavy.

Testis is not palpated separately from the swelling

Testicular sensation is absent.

Epididymis and spermatic cord – normal

Left side testis is normal

Penis is in midline and normal

No regional lymph nodes palpable.

Other system Examination

Differential Diagnosis

Testicular tumour

Chronic Hematocele

Chronic hydrocele

Specific investigation

Testicular tumour markers – LDH ,HCG ,AFP

Ultrasound scrotum

Testicular biopsy – Inguinal approach

Routine Investigations

Metastatic workup – X ray chest/ CT chest/ CT abdomen

Treatment - Orchiectomy / Radiotherapy / Chemotherapy

UNDESCENDED TESTIS

Mr. Prakash 19 years / Male College Student from Royapuram Came with

Chief Complaints

Absence of right testis since birth.

Swelling in the right inguinal region for the past 6 months

Pain in the swelling for past 1 week.

History of presenting illness

H/O swelling in the Right inguinal region for past 6 months

Insidious in onset ,Progressive in nature

H/O pain in the right inguinal region which is intermittent, dull aching,aggravated on walking and relieved on compression

No H/O swelling in root of penis, thigh and perineum.

No H/O chronic cough

No H/O constipation or difficulty in micturation

No H/O previous surgeries

No H/O blood transfusions

Past history

Personal history

Family history

General examination

Vital signs

Local examination - Inspection

Swelling of size 8*4 is seen on the Right inguinal region

Pyriform in shape

Extends 7 cm from ASIS to groin

Surface is smooth

Skin over the swelling is normal

Cough impulse present

No scar or sinuses

Scrotum is well developed on left side and poorly developed on Right side

Palpation

Not warmth, non tender.

Deep ring occlusion test – swelling does not appears. cough impulse felt.

Right sided testis is not palpable in the scrotum.

Left sided testis is palpable.

Penis is normal.

No organomegaly.

Other system Examination

Diagnosis

A case of right sided undescended testis associated with right sided incomplete indirect inguinal hernia.

Specific Investigation

To find out the location of testis

Ultrasonogram of inguinal region & scrotum

Routine investigations

Treatment

ORCHIDOPEXY

Adequate mobilisation of the testis and spermatic vessels.

Ligation of the associated hernial sac.

Adequate fixation of the testis in the dependent portion of the scrotum.

CA PENIS

Mr.Moorthy 42 year old male from Thiruvallur working as a security came with complaints of

Ulcer Over The Penis For Past 5 Months

History of presenting illness

The patient was apparently normal before 5 months. Then he developed,
Ulcer over the penis for the past 5 months

Insidious in onset,progressive in nature

not associated with pain

The ulcer was initially in the inner prepuce and now it had grown to involve the whole of Glans.

H/O Discharge from the ulcer for the past 15 days

Purulent in nature

Associated with foul smell,not blood stained

No H/O Trauma

No H/O Fever

No H/O Burning micturition

No H/O Micturition difficulties

No H/O Hematuria

No H/O Loss of weight /Loss of appetite

No H/O Bone pain/ Jaundice /Breathlessness

Past history

Personal history

Family history / Rx History

General examination

Vital signs

Local examination - Inspection

SITE- Glans Penis,SIZE 4X3 cm,Irregular in shape

Margins- ill defined,Edge- raised and everted

Floor- unhealthy tissue

External urethral meatus is seen.

Surrounding area- edematous.

Dilated veins seen in dorsum of penis

1x1 cm ulcer present on the ventral surface of penis

Palpation

Not tender

Inspectory findings are confirmed

size – 4x3 cm.

The ulcer extends from 5 cm from the root and involves the whole of glans.

Irregular in shape ,Base of ulcer- indurated induration extends 1cm beyond the margin

The ulcer does not bleed on touch

Examination of lymph nodes

Right inguinal region- Node of size 1x1cm, non-tender, firm in consistency, mobile.

Left inguinal region- Node of size 1.5x1cm,non-tender, firm in consistency, mobile.

Other system Examination

Specific investigation

Wedge biopsy of the ulcer.

FNAC of lymph nodes.

Routine and Staging investigation (Chest x-ray & USG and CT abdomen)

Diagnosis

A case of non-healing ulcer involving glans penis with bilateral palpable inguinal lymph nodes probably Carcinoma Penis at stage III.

Treatment - Partial penectomy with oral antibiotic therapy for the inguinal lymph nodes for 3 weeks to exclude infective etiology. If the nodes have not resolved at end of 3 weeks to proceed with inguinal node dissection.

FIBROADENOMA

Mrs. Saritha Devi 24 Year old, coming from Moolakadai, working in an export company came with chief complaint of **Lump in the Right breast for the past four months.**

History of Presenting illness:

Patient was apparently normal before 4 months. Then she noticed a lump in the Right breast while taking bath. Duration of 4 months.

Insidious in onset, Slowly progressed to attain present size.

Not associated with pain.

No history of trauma.

No history of fever.

No history of discharge from nipple.

No history of nipple retraction.

No history of loss of weight or loss of appetite

No history of bone pain.

No history of jaundice.

No history of breathlessness and cough with hemoptysis.

Past history (? Recurrence)

Personal history

Family history / Rx History

General examination

Vital signs

Local examination of Breast – Inspection (After consent/Privacy/Exposure)

Patient in sitting position with arms by side of the body.

Rt Breast is normal in position, size and shape on comparison with the opposite breast.

No visible lump on inspection.

Skin over the breast is Normal,

No scars, No dilated veins, No dimpling, No ulceration

No peau d'orange appearance

Nipple – Single, normal in position, size and shape. No cracks or fissures.

Areola – Normal in colour and size. No cracks/fissures.

Arms and Thorax – Normal

Supraclavicular region – No fullness.

Patient with arms raised above the head:

Both breasts rise equally, No puckering

Inframammary region seen

No retraction of nipple, No peau d'orange appearance.

Patient on leaning forward :

Breasts fall equally on both sides.

Palpation :

Not warm, Not tender.

A single lump in the upper outer quadrant of the Right breast.

Size of 3x3 cms., globular in shape.

Surface is smooth. Margins are regular, well-defined.

Firm in consistency.

Intrinsic mobility is present.

Not fixed to skin/Underlying Muscles/Chest wall

No palpable axillary lymph nodes.

Diagnosis - A case of Fibroadenoma in the upper outer quadrant of the Right breast.

Specific investigations

Fine needle aspiration Cytology

Ultrasound

Routine Investigations

Treatment : Excision biopsy.

GANGLION CYST

A 25 year old Female MS.SHYLA ,coming from Korukkuppet , came with chief complaints of

Swelling in the right wrist for the past 6 months.

History of presenting illness:

Patient was apparently normal before one year. Then she developed a swelling in the right wrist for which she was injected once following which the swelling subsided.

After 6 months , the swelling recurred in the same site.

It was insidious in onset

Started as a small swelling gradually progressed to attain the present size.

Not associated with pain.

No H/o fever

No H/o trauma

No H/o any other swelling anywhere in the body.

No H/o loss of weight & appetite.

No H/o restriction of movements.

No H/o secondary changes in the swelling

Local examination:

INSPECTION

Single swelling on the Lateral aspect of the right wrist joint 1.5x1cm in size spherical in shape well defined margins

Surface : smooth

Skin over the swelling is normal

No visible pulsation

No scars, sinus or dilated veins

No secondary changes over the swelling

Palpation:

Not warm ,Not tender

Inspectory findings like site size shape surface are confirmed

Margins are well defined

Skin over swelling is pinchable

Surface is smooth

Tensely cystic in consistency

Mobility : Mobile only on the horizontal axis

Vertical Mobility is restricted normally and when the wrist joint is extended

Slip sign negative

Fluctuation is **positive**.

Transillumination is **positive**.

Not compressible or reducible.

Regional lymph nodes are not palpable

No neurovascular deficit

Provisional diagnosis :

A case of ganglion in the right wrist

Specific USG

General Investigation

Treatment

Excision biopsy